Evaluating the Mental Health Service Delivery System for Refugees

School of Social Work
COLLEGE OF EDUCATION + HUMAN DEVELOPMENT

Patricia Shannon, PhD*, Gregory Vinson, PhD**, Tonya Cook, MSW*, Kao Nou L. Moua, MSW*, Evelyn Lennon, MA, MSW**

*School of Social Work, **Center for Victims of Torture

University of Minnesota
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Introduction

32,246 refugees from countries such as Burma, Bhutan, Somalia, Ethiopia, and Iraq have arrived to the Twin Cities in the past 10 years [1]. A recent meta-analysis reported that in 80,000 refugees surveyed, 30.6% report suffering from posttraumatic stress disorder (PTSD) and 30.8% from depression [2]. The lack of mental health resources has been identified as a reason for not screening the mental health of newly arriving refugees [3].

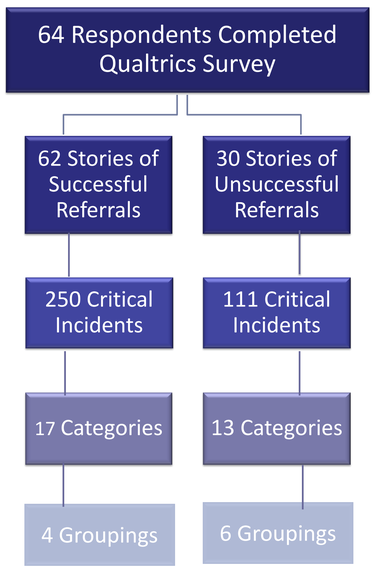
Research Questions

1) What makes mental health referrals successful and unsuccessful for MN's refugee communities?

2) What are likely interventions at individual and health care system levels for improving access and the delivery of culturally appropriate mental health services for refugee populations?

Methods

This study combined a Community Based Participatory Research approach with Critical Incident Technique. It evaluated the effectiveness of mental health services for refugees from the perspectives of: (a) providers working with refugees; (b) primary insurance-based network providers; and (c) refugee community leaders. Survey participants gave stories of successful and unsuccessful referrals of refugees for mental health services.



"Describe in detail what happened when you made a successful or unsuccessful referral of a refugee for mental health services".

Stories containing multiple behavioral examples minimally edited into discrete critical incidents (CIs).

10 trained sorters knowledgeable about refugees used a pile sort method to sort CIs into categories of their choosing. Principle components analysis yielded categories.

Hierarchical Factor Analysis yielded final groupings.

Results

Factors that Contribute to Successful MH Referrals for Refugees

	N	% Variance*
ACTIVE COORDINATION OF CARE		
Provider Actively Coordinates Care	30	11.26
Provider Makes a MH Referral	20	6.81
Provider Assists with Scheduling MH Appointment	19	6.68
Refugee Engages Crisis System to Access MH Services	13	4.38
Active/Ongoing Case Management by Health Plan Staff	8	2.71
TRUST		
Provider Identifies the MH Needs of a Refugee Client	27	9.40
Provider has Embedded MH or Referral Coordinators	12	3.78
Referrals Initiated by Client or Family	8	3.54
(Non-Health) Provider Established Trusting Relationship Prior to Referral	4	2.28
Health Provider Established Trusting Relationships Prior to Referral	4	1.54
SYSTEM RESPONDS TO NEEDS OF REFUGEES		
Psychoeducation Provided about MH Services	21	7.74
Provider Uses and Arranges Interpreter Services	20	6.87
Provider Arranges, Educates About, Provides Transportation	17	6.07
Provider Follows-Up to Ensure the Success of the Referral	15	3.95
CULTURALLY RESPONSIVE CARE		
Provider Refers to or Provides Culturally Appropriate Care	12	4.27
Provided Service in Client's Home	11	3.86
Provider Actively Connects Clients to Resources to Meet Additional Needs	9	2.18
TOTAL	250	87.34

Factors that Contribute to Unsuccessful MH Referrals for Refugees

	N	% Variance*
CULTURAL BARRIERS TO REFUGEES ACCESSING CARE		
Client Declines Services or Finds them Culturally Inappropriate	24	21.00
Clients' Religious Beliefs Conflicted with Recommended Treatment	4	4.05
Client Lacks Knowledge about MH Services	4	3.01
COORDINATION BARRIERS		
Provider Fails to Coordinate Care in an Adequate or Timely Manner	17	12.69
MH Service System Fails to Provide Transportation	10	9.04
Provider or Client Unable to Initiate First Appointment	4	2.68
PROVIDER BARRIERS: UNWILLING OR UNABLE TO SERVE REFUGEES		
Provider Declined to Make or Accept a Refugee MH Referral	8	6.01
Provider Lacks Competence to Work with Refugees or Trauma	3	3.11
MH SYSTEM BARRIERS: LACK OF AVAILABLE CARE		
Provider Unavailable or Wait for MH Appointments is too Long	8	7.63
Provider Thinks the Client is Too Sick or Unstable	2	2.78
Client Lacks Health Insurance or is Ineligible for Services	4	2.70
INTERPRETER BARRIERS		
Provider Fails to Utilize Appropriate Interpreter Services	21	17.78
PROVIDER MAKES INAPPROPRIATE REQUESTS OF ETHNIC COMMUNITY MEMBERS		
Mainstream Providers Make Inappropriate Requests of ECBO Staff	2	1.77
TOTAL	111	94.25

* % Total Variance (PCA) refers to the percentage of variance accounted for by the commensurate component in the principle components analysis

Sample

Professional Roles of Respondents	*N*
Mental Health (MH) Provider in Private Practice, Social Service Agency, Primary Care, or MH Agency Setting	19
Health Plan Staff	9
Ethnic Community-Based Organization (ECBO)	7
Refugee Resettlement Agency	7
Provider in Primary Care or Public Health Clinic Setting	5
Social Service Agency or Legal Clinic	5
Public Health Nurse	4
Social Worker, Care Coordinator, Case Manager in Primary Care or MH Agency Setting	3
State or County Health or MH Agency	3
School Counselor or ELL program Staff	2
TOTAL	64

Participants shared stories about a diverse sample of refugees, including: Cambodian, Congolese, Eritrean, Ethiopian, French-speaking African, Hmong, Iraqi, Karen (Burma), Laotian, Liberian, Russian, Somali, Ugandan, and Vietnamese.

Recommendations

Participant and Steering Committee Recommendations

- Improve system navigation and care coordination for refugees
- Provide trainings and incentives for providers and system navigators to work with refugees and trauma
- Provide MH training for frontline workers (e.g. social service providers) and interpreters
- Provide psychoeducation to refugee communities
- Improve access to care and timely responses
- Increase capacity of SPMI and emergency services to serve refugees

References & Acknowledgements

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